

OpWell Concierge™

Clinical Evidence Brief & Operational Value Analysis

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Executive Summary

OpWell Concierge is an independent, telehealth-based perioperative specialty service that translates multicenter RCT evidence from Enhanced Recovery After Surgery (ERAS), Perioperative Surgical Home (PSH), and prehabilitation literature into a comprehensive optimization model across two distinct phases: preoperative preparation and structured 30-day post-discharge monitoring. OpWell does not participate in intraoperative care.

Evidence Table: Clinical and Operational Claims

Clinical Claim	Effect Estimate	Certainty	Primary Source
Complication reduction via exercise prehabilitation	OR 0.50 (0.39–0.64); ~50% RR	Very Low*	Mclsaac et al., BMJ 2025; 186 RCTs, n=15,684
Complication reduction via exercise (confirmatory)	OR 0.54 (0.44–0.67)	Moderate	Ramic et al., Anesth Analg 2025; 99 RCTs, n=8,222
Hospital LOS reduction via ERAS pathways	MD –1.88 days (–2.81 to –0.95)	Moderate	Sauro et al., JAMA Netw Open 2024; 74 RCTs, n=9,076
Complication reduction via ERAS pathways	RR 0.71 (0.59–0.87)	Mod-High	Sauro et al., JAMA Netw Open 2024
Cost savings — abdominal ERAS	–\$5,109 per case (–\$5,852 to –\$4,366)	Moderate	Visioni et al., Ann Surg 2018; 39 studies, n=6,511
Anesthesiologist-led PSH operational impact	Decreased LOS, ICU use, opioids; increased home discharge	Low	Cline et al., J Clin Anesth 2020; 11 studies
Cost-effectiveness of prehabilitation	64% (16/25) support cost-effectiveness	Low**	Rombey et al., BMC Med 2023
ED visit reduction via mHealth monitoring	OR 0.42 (0.23–0.79); ~58% reduction	Low	Dawes et al., BJS 2021; 45 studies
Readmission reduction via mHealth	OR 0.47 (0.29–0.77); ~53% reduction	Low	Dawes et al., BJS 2021
Readmission reduction via digital health	RR 0.67 (0.58–0.78); ~33% reduction	Low-Mod	Grygorian et al., JAMA Netw Open 2024;7(4):e244963
Preoperative patient education — LOS	MD –1.01 days (–1.86 to –0.16)	Moderate	Hall et al., J Am Coll Surg 2025; 40 RCTs, n=4,113
QoR-15 psychometric validity	Cronbach alpha=0.85 (0.83–0.87); r=0.97	High	Myles et al., Br J Anaesth 2022;128(6):1029-1039
Same-day cancellation reduction	12.3% → 8.4% rate; \$150,200 savings	Moderate	Wongtangman et al., Anaesthesia 2024;79(6):618-627
Preoperative assessment → cancellation reduction	AOR 0.55 (0.43–0.72)	Moderate	Tan et al., Int J Surg 2019

* Robust after exclusion of high risk-of-bias trials. ** Publication bias risk noted by authors.

Critical Nuance: Post-Discharge Monitoring Requires Structured Technology

A systematic review by Boggan et al. (2025) of 7 randomized trials (n=7,075) demonstrated that simple, single-contact telephone calls within 7 days of discharge produced no reduction in 30-day readmissions or ED visits. The significant reductions in readmissions and ED visits (OR 0.42–0.67) are strictly specific to structured, technology-enabled, multi-contact monitoring platforms—the exact clinical standard operationalized by OpWell Concierge.

OpWell Surgery School: POQI Consensus Alignment

Every referred patient completes a 23-module digital preparatory framework aligned with 2025 POQI recommendations across five therapeutic domains:

- Procedure-Specific: Personalized pathways for 8 major specialties
- Behavior Change: Targeted nutritional, fitness, and cessation coaching
- Psychological: Anxiety mitigation and clinical visualization
- Practical Preparation: Home configuration and caregiver alignment
- Expectations: Comprehensive 30-day week-by-week recovery roadmap

B2B Integration and Clinical Continuity

OpWell Concierge operates exclusively in the preoperative and post-discharge phases as a collaborative extension of existing clinical practice. It does not provide intraoperative services, alter surgical plans, manage technical operative complications, or replace standard postoperative office visits.

Transparency Statement on Evidence Certainty

Strongest Evidence (Moderate-High): ERAS pathways consistently reduce LOS by ~1.9 days and complications (RR 0.71). Strong Evidence (Very Low–Moderate): Exercise prehabilitation demonstrates consistent 50–54% complication reduction. Moderate Evidence (Low–Moderate): Post-discharge monitoring shows the strongest signal from structured, technology-enabled platforms (OR 0.42–0.67). Conclusion: The OpWell clinical framework is structurally aligned with moderate-to-high certainty evidence across its core therapeutic domains, delivering a rigorously validated, evidence-based asset to surgical programs.

References

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